Which treatment this guide covers NCCP 00219a is first-line EGFR-mutated advanced NSCLC; 00219b is switch-maintenance after stable disease following first-line chemotherapy; 00219c is after failure of at least one previous chemotherapy regimen. Global NCCP eligibility requires a validated activating EGFR mutation in each setting. These are advanced-disease settings, not adjuvant treatment.

NCCP 00219 · VERSION 6

Erlotinib monotherapy

The usual dose is 150 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Diarrhoea and dehydration

☐ Serious lung inflammation

☐ Eye inflammation

☐ Skin, nail and mouth changes

☐ Liver injury

☐ Rare bowel perforation and serious skin reactions

Discuss the aim of advanced cancer control, expected benefit and uncertainty, alternatives including not proceeding, bowel and skin effects, serious lung/liver problems, fertility and pregnancy, daily treatment, monitoring and your right to reconsider.

Individual benefit, alternatives and concerns

Agreed checks and next review

NCCP specifies blood count, kidney and liver tests every 60 days. Diarrhoea or dehydration needs extra kidney and electrolyte tests, including potassium. Liver problems or interacting medicines can require closer tests. Scans follow the Consultant’s plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.26 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending